

Slide 6 Reading Notes

Ethical considerations | Meds Consulting LLC

Opening

This slide explains the ethical considerations for NHN's readmission analytics. The goal is to make sure the risk model and dashboard support better care without creating unfair, opaque, or noncompliant decisions.

The four guardrails at the top are fairness, privacy, governance, and compliance. These are the standards we should apply before NHN scales the model beyond a pilot.

1. Algorithmic bias

The first issue is algorithmic bias. A readmission model learns from historical data, and historical data can reflect unequal access to care, documentation gaps, or operational differences across patient groups.

NHN should monitor model performance across relevant groups such as age, diagnosis, discharge type, and insurance where appropriate. The key question is whether the model makes more errors for some groups than others.

2. Customer privacy

The second issue is patient privacy. The analysis uses patient-level health, utilization, care coordination, and financial fields. These are sensitive fields, so access should follow the minimum necessary principle.

That means only teams with a clear care, reporting, or governance need should access patient-level data. Dashboard views should use role-based access and avoid exposing unnecessary identifiers.

3. Data governance

The third issue is data governance. Earlier slides showed missing follow-up status and financial anomalies. If those issues are not governed, the model and ROI estimates can drive the wrong decision.

NHN should define data owners, validation rules, refresh timing, change logs, and escalation steps. Governance is especially important before model refreshes or monthly executive reporting.

Governance need	Control
Data quality	Validate missing values, outliers, and financial anomalies
Model use	Document purpose, limitations, and approved users
Accountability	Assign owners for refresh, dashboard definitions, and escalation

4. Transparency

The fourth issue is transparency. Clinicians and operations teams need to understand why a patient receives a high-risk flag. If the model acts like a black box, teams may not trust it or may use it incorrectly.

The model output should show key drivers in plain language, such as prior admissions, chronic condition burden, discharge type, or unknown follow-up status. The score should guide review, not replace clinical judgment.

5. Regulatory compliance and responsible AI

The fifth issue is regulatory compliance. NHN should review data use, access, retention, and reporting with compliance leaders before scaling the dashboard or risk model. HIPAA and internal policy should guide what data is used and who can see it.

Responsible AI means the model remains a decision-support tool. Care teams should keep final judgment over patient outreach, discharge decisions, and care coordination actions. Finance should also validate assumptions before treating ROI estimates as budget commitments.

6. Decision standard

The decision standard is simple: use the model only when fairness checks, privacy controls, data governance, transparency, and compliance review are in place.

This protects patients, supports clinicians, and makes the recommendation more credible for leadership.